



Dementia Communication Toolkit

VA



U.S. Department of Veterans Affairs

Veterans Health Administration
Ralph H. Johnson VA Health Care System

9 CAREGIVING TIPS

Compassionate Communication with the Memory Impaired

© by Liz Ayre: a volunteer of the Alzheimer's Association Orange County Chapter and former caregiver
From the Alzheimer's Association website: <http://www.alzsf.org/abtalz/compcomm.asp>

Don't

- Don't reason, don't argue
- Don't confront
- Don't remind them they forget
- Don't question recent memory
- Don't take it personally

Do

- Give short, one sentence explanations
- Allow plenty of time for them to take in what you are saying, then triple it
- Repeat instructions of sentences the same way
- Get rid of "but" when you speak and use "nevertheless"
- Avoid pushing – try again later
- Agree with them or distract them to a another subject or activity.
- Accept the blame when something's wrong (even if it's not real)
- Leave the room to avoid fighting or arguing if you must
- Respond to the feelings rather than the words
- Be patient, cheerful, and reassuring. Do go with the flow
- Practice 100% forgiveness. Memory loss may worsen daily
- My appeal to you: Please elevate your level of giving and being gracious

Remember

You can't control memory loss, only your reaction to it. *Kindness in word and deed will significantly heighten quality of life.*

They are not crazy or lazy. They say and do normal things for a memory impaired person. If they were trying to wear you out on purpose, they would have been found to have a mental health disease and not dementia. *Forgive them...always.* For example: They don't hide things; they protect them in safe places...and then forget. *If they say you took or stole something don't take it personally.*

Their health problem is memory loss. Asking them to remember is like asking a blind person to read. ("Did you take your pills?" "What did you do today?") *Don't ask and don't test memory!* A loss this big also takes away the capacity to reason. Expecting them to be able to reason or to accept your final decision is not likely. ("You need a shower." "Day care will be fun." "You can't live alone.") *Don't try to reason or change their mind. Give a short statement for why or search for other options.* Memory loss

produces random emotions, thoughts, and actions which you can reduce by working on all issues peacefully. *Don't argue, correct, contradict, confront, blame, or insist.*

Reminders are rarely kind to someone. They tell the patient how disabled they are—over and over again. Reminders of the recent past imply, "I remember, I'm okay; you don't, you're not." Ouch! Refer only to the present or the future. If they're hungry, don't inform them they ate an hour ago, offer a snack or set a time to eat soon. They may ask the same question repeatedly, believing each time is the first, so practice kindness in your response as if it's the first time. Some days they seem like they may be better, but they're not. They live in a different reality so a reminder won't bring them into yours. Note: For vascular dementia, giving clues may help their recall. If it doesn't work, be kind...don't remind.

Ethical problems may occur. If the patient thinks a dead spouse is alive, and truthful reminders will create sadness, what should you do? To avoid distress, try these ways of kindness: Distract to another topic, start a fun activity, or reminisce about their spouse, "I was just thinking about _____. How did you two meet?" You might even try, "He's gone for a while. Let's take our walk now."

Open-ended questions like "Where shall we go?" or "What do you want to eat/wear/do?") can be very hard and create anxiety. Give them a simple choice between two items or direct their choice, "You look great in the red blouse."

They may be scared all the time. Each patient reacts differently to fear. They may become passive, uncooperative, hostile, angry, agitated, verbally abusive, or physical. They may even do them all at different times or go back and forth between them. Anxiety may compel them to "shadow you" or follow you everywhere. Anxiety compels them to resist changes in routine, even pleasant ones. Your goal is to reduce anxiety whenever possible. Also, they can't remember what you say to help them feel safe and calm so keep saying them.

Examples of Kind Speak

Don't reason

Patient: "What doctor's appointment? There's nothing wrong with me."

Don't: (reason) "You've been seeing the doctor every three months for the last two years. It's written on the calendar, and I told you about it yesterday and this morning."

DO: (short explanation) "It's just a regular check-up." (accept blame) "I'm sorry if I forgot to tell you."

Don't question recent memory

Patient: "Hello, Mary. I see you've brought a friend with you."

Don't: (question memory) "Hi Mom. You remember Eric, don't you? or What did you do today?"

DO: (short explanation) "Hi Mom. You look wonderful! This is Eric. We work together."

Don't argue

Patient: "I didn't write this check for \$500. Someone at the bank is forging my signature."

Don't: (argue) "What? Don't be silly! The bank wouldn't be forging your signature."

DO: (respond to feelings) "That's a scary thought."

(reassure) "I'll make sure they don't do that."

(distract) "Would you help me fold the towels?"

Don't take it personally!

Patient: "Who are you? Where's my husband?"

Don't: (take it personally) "What do you mean—who's your husband? I am!"

DO: (go with the flow and reassure) "He'll be home for dinner."

(distract) "How about some milk and cookies? Would you like chocolate chip or oatmeal?"

Don't confront

Patient: "Nobody's going to make decisions for me. You can go now...and don't come back!"

Don't: (confront) "I'm not going anywhere, and you can't remember enough to make your own choices."

DO: (accept blame or respond to feelings) "I'm sorry this is a tough time."

(reassure) "I love you and we're going to get through this together."

(distract) "You know what? Don has a new job. He is really excited about it."

Do repeat exactly

Patient: "I'm going to the store for a newspaper."

Don't: (repeat differently) "Please put your shoes on...you'll need to put your shoes on."

DO: (repeat exactly) "Please put your shoes on...please put your shoes on."

Don't remind them they forget

Patient: "Joe hasn't called for a long time. I hope he's okay."

Don't: (remind) "Joe called yesterday and you talked to him for 15 minutes."

DO: (reassure) "You really like talking to Joe, don't you?" (distract) "Let's call him when we get back from our walk."

Do eliminate 'but'; substitute 'nevertheless'

Patient: "I'm not eating this. I hate chicken."

Don't: (say 'but') "I know chicken's not your favorite food, but it's what we're having for dinner."

DO: (say 'nevertheless') "I know chicken's not your favorite food, (smile) nevertheless I would be thankful if you would eat a little bit."

Maintaining a routine and modifying the environment

- Have a routine. Knowing what to expect can be comforting the Veteran. Post a schedule in Veterans room that they can see.
- Schedule more challenging activities (like bathing) when the person is likely to be at their best.
- Schedule pleasurable activities, like music, singing, dancing. Order a meal with their favorite foods. Give enough time for activities.
- Other activities to consider: walking, talking on the phone, having a snack, 'housework' in room, praying, other religious activities.
- Don't over schedule or overstimulate. Reduce clutter, noise, and distraction. If the current location is too distracting, may consider moving to quieter space on the unit.
- Schedule meaningful activities. Find ways that the Veteran can contribute, e.g. folding towels or washcloths. Allow the Veteran to take advantage of their remaining social skills.
- Try activities to stimulate the senses: finger painting (using non-toxic paints), popping bubble wrap, build or buy a "busy board," try a "fidget" blanket or other sensory toys.

Responding to aggression and potentially unsafe behaviors

- Watch for early signs of agitation or aggression, and respond before things escalate.
- Keep track of behaviors, and try to identify causes or triggers of the behaviors.
- Don't confront or argue. Don't make sudden movements.
- Try distraction. Shift to another activity. Music may be soothing. They might forget what they were angry about.
- Give a Veteran who paces a lot a safe place to walk. Make sure they are hydrated and eating snacks. Get them good shoes.
- Limit access to dangerous items, such as sharp objects.

Responding to hallucinations and delusions

- Don't argue or try to reason. Discuss the underlying emotion, which is usually fear (or maybe anger).
- If the Veteran is saying that the object is missing or has been stolen, try to search for the object. Then talk about another object you found, e.g., photo album (distraction).
- Turn off the TV or computer when there are violent or upsetting programs or images.
- If the person is "hearing things" (auditory hallucinations), check for noises that they could be misinterpreting.
- If a Veteran is "seeing things" (visual hallucinations) make sure there is adequate lighting. Look for shadows or reflections that they could be misinterpreting. Cover or remove mirrors (they could think their reflection is a stranger).

Responding to problems with sleep

- Schedule enough activities during the day to stay busy. Address daytime loneliness as boredom. Encourage Veteran to exercise daily.
- Create a daily routine with the same waking up time and bedtime each day.
- Napping is OK, as long as it is not too late (for example, 2 p.m. or later) and not too long (more than one hour). It may be helpful to schedule one or two rest periods each day.
- If the Veteran drinks coffee or tea, switch to decaffeinated versions.
- Before bedtime, create a calm and soothing environment. Turn off the TV. Soothing music may help. Read to the Veteran.
- Keep a nightlight on in the bedroom, hallways, and bathroom.
- If the Veteran is up at night: Approach them calmly. See if they need something. Gently remind that that it is bed time, without arguing.

Gain Awareness About Dementia

A Positive Approach to care

1. [How Dementia Affects Language Skills - Bing video](#)
2. [Vision Changes with Dementia - Bing video](#)
3. [Bathing and Dementia - with Teepa Snow of Positive Approach to Care \(PAC\) - Bing video](#)
4. [Care Partner Support Tips: From Moving to Calm in the Ruby GEMS State - YouTube](#)
5. [How to Help a Person Living with Dementia Brush their Teeth - with Teepa Snow - YouTube](#)
6. [Connecting Through Music-YouTube](#)
7. [Meaningful Activities - YouTube](#)
8. [Challenging Behavior - YouTube](#)

Additional videos about dementia:

<https://teepasnow.com/resources/pac-videos-about-dementia/>

*free online basic dementia course:

<https://shop.teepasnow.com/product/dementia-perspectives-seeing-it-from-the-other-side-online-course/>

[All About Dementia - YouTube](#)